INTRODUCTION

NovaBirth is building an AI-assisted ultrasound tool that gives clinicians real-time measurements of cervical dilation and fetal head position during labour

My role: Solo UI/UX designer

Deliverable: Stakeholder-facing mobile app prototype

Timeline: 3 weeks

Constraints: Concept-stage only, no access to clinicans

THE PROBLEM

Manual vaginal exams during labour are subjective, invasive, and uncomfortable

Currently, intrapartum assessment relies on a finger-based vaginal exam.

A clinician manually feels the cervix with gloved fingers and estimates cervical dilation against a trained, subjective mental scale.

> **NovaBirth's Objective:** Rather than attempting to replace this exam entirely, NovaBirth aims to reduce its required frequency by supplementing clinical judgment with non-invasive, objective ultrasound measurements.

DESIGNING WITHOUT CLINICIANS

I couldn't observe a labour ward or interview a midwife, so I treated the research as a set of questions to answer from secondary sources

TASK FLOW & INFORMATION ARCHITECTURE

This research directly shaped my task flow.

LOW-FIDELITY WIREFRAME

I designed the lo-fi wireframe, then reviewed it critically

I found two issues, highlighted in the image below.

I moved Scan to a persistent, globally accessible button and updated the Information Architecture to reflect it.

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Getting the placement right took a few iterations.

An early version overlapped its own label with the tab bar underneath it, and a later version collided with the content behind it.

I settled on a floating button placed outside the tab bar entirely.

Added an option for manual entry

HIGH-FIDELITY PROTOTYPE

I designed within NovaBirth's existing brand colors throughout the app, as per direction from the team

Reflection

Designing without clinician access forced an intellectual honesty I wouldn’t have developed otherwise. I couldn’t lean on “a user told me this,” so every choice had to earn its place through research and logic.

What I'm proudest of isn't any single screen. It's that I never let the prototype get ahead of what I could actually justify. When I made a decision without clinical confirmation, I grounded it in research. That restraint is harder than it sounds when you're excited about a product and want to show something polished.

The next step is getting this in front of real clinicians, so the design is built with the people who’ll use it, under the pressure it was meant to survive.